

Clinical Information for Medical Necessity Review

CMS Policy Hierarchies & Application

Guideline Definitions

GENERAL

Preface	Abdomen
Breast	Cardiac
Cardiac Implantable Devices (CID)	Chest
Head	Musculoskeletal
Neck	Oncology
Pelvis	Peripheral Nerve and Neuromuscular Disorders (PNND)
Peripheral Vascular Disease (PVD)	
Spine	

PEDIATRIC

Pediatric Abdomen	Pediatric Cardiac
Pediatric Chest	Pediatric Head
Pediatric Musculoskeletal	Pediatric Neck
Pediatric and Special Populations Oncology	
Pediatric Pelvis	
Pediatric Peripheral Nerve and Neuromuscular Disorders (PNND)	
Pediatric Peripheral Vascular Disease (PVD)	
Pediatric and Special Populations Spine	

Clinical Information to Establish Medical Necessity Guideline

Guideline

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The clinical guidelines apply an evidence-based approach to evaluate the most appropriate medically necessary care for each individual. This evaluation requires submission of legible medical records pertinent to the test or treatment being requested by the provider.

If the medical records provided do not provide sufficiently detailed information to understand the individual's current clinical status or cannot be read, then medical necessity for the request cannot be established and cannot be approved.

Specific elements of an individual's medical records commonly required to establish medical necessity include, but are not limited to:

- Virtual or in-person clinical evaluation which includes a detailed history and physical examination since the onset or change in symptoms
- Laboratory studies
- Imaging studies
- Pathology reports
- Procedure reports
- Reports from other providers participating in treatment of the relevant condition

Medicaid and Medicare Hierarchy and Application

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Medicaid

Medicaid Hierarchy

1. Medicaid state-specific policy
2. These evidence-based guidelines or the appropriate alternative guideline utilized by a program/health plan in place of these guidelines
3. Early and Periodic Screening, Diagnostic and Treatment (EPSDT) guidelines should also be reviewed for individuals under 21 years of age as applicable for coverage determination

Application of Medicaid Policy

The state Medicaid policy will be reviewed first to determine if the information provided is instructive to the clinical case at hand. For the purpose of this policy, sufficient clinical criteria to render a medical necessity decision is defined as the presence of a state Medicaid policy that addresses the service/procedure/test/equipment and the member condition (indication) and supplies sufficient clinically relevant detail to be instructive to the case. See the numbered items below for specific guidance:

1. State Medicaid policy addresses clinical scenario: (service/procedure/test/equipment AND member condition AND Medical Necessity criteria):
 - a. Medicaid policy will be utilized and cited for determining medical necessity
 - b. If specific medical necessity criteria are present in the state policy, but are limited or less detailed than eviCore or alternative guideline, the state policy would still be applied and cited, as this would be considered sufficient information to render a decision
2. State Medicaid policy addresses service/procedure/test/equipment, but does NOT cover member condition in question or provide specific medical necessity criteria for this indication and does not contain relevant clinical information to be instructive to the case/clinical scenario, eviCore guidelines or alternative guidelines, as appropriate, will be utilized and cited
3. State policy exists for service/procedure/test/equipment AND member condition but NO medical necessity criteria exist for this indication. The state policy would still be instructive to the case clinical scenario and Medicaid will be utilized and cited.

- 4. (Example: Service is requested to address a member condition and the state policy indicates that the service is indicated for this member condition without providing specific criteria regarding under which circumstances the service would be indicated for this condition. This is considered instructive, and the state policy would be applied/ cited.)
- 5. If no clinical information is provided with a request, the above hierarchy is still applied. If there is an applicable state-specific Medicaid policy, the Medicaid policy would be utilized and cited to request clinical information/documentation. If there is no applicable state-specific Medicaid policy, eviCore guidelines or alternative guidelines, as appropriate, will be utilized and cited

Note: The scope of this policy is to outline the default order in which policy sources will be used during a delegated medical necessity determination. The Medicaid hierarchy outlined here may be superseded by Plan- and State-specific Hierarchy policies, where applicable, based on contracts with a state Medicaid entity.

Application and citation of policy

State Medicaid Policy exists	State Medicaid policy addresses service/ procedure/test/ equipment	State policy addresses member condition and/ or is instructive to the case	State Medicaid Policy provides specific medical necessity criteria relevant to service and member condition	Application and Citation of Policy
Y	Y	Y	Y	State Medicaid policy applied and cited
Y	Y	Y	N	State Medicaid policy applied and cited
Y	Y	N	N	EviCore criteria applied and cited

State Medicaid Policy exists	State Medicaid policy addresses service/ procedure/test/ equipment	State policy addresses member condition and/ or is instructive to the case	State Medicaid Policy provides specific medical necessity criteria relevant to service and member condition	Application and Citation of Policy
Y	N	N	N	EviCore criteria applied and cited

Medicare Policy

CMS Coverage Hierarchy: Medicare Advantage Medical Policy Development and Application

Medicare Advantage medical policies identify the clinical criteria for determining when medical services are considered 'reasonable and necessary' (medically necessary). Medicare Advantage plans are required by CMS to provide the same medical benefits to Medicare Advantage members as original Medicare.

Medicare Advantage plans must comply with national coverage determinations (NCD), local coverage determinations (LCD), and general coverage and benefit conditions included in traditional Medicare laws. This includes criteria for determining whether an item or service is a benefit available under traditional Medicare. When coverage criteria are not fully established in Medicare statute, regulation, NCD, or LCD, Medicare Advantage organizations may create publicly accessible internal coverage criteria that are based on current evidence in widely used treatment guidelines or clinical literature.

If additional criteria are needed to interpret or supplement generalized provisions within an NCD, LCD or other Medicare coverage policy, or there is flexibility allowed in the NCD/LCD, or there is no applicable NCD, LCD, or LCA (used in concert with an LCD) available to determine medical necessity, then other evidence-based criteria may be applied.

Coverage criteria are not fully established when (42 CFR 422.101(6) (i) :

(i) Coverage criteria not fully established. Coverage criteria are not fully established when:

- (A) Additional, unspecified criteria are needed to interpret or supplement general provisions in order to determine medical necessity consistently. The MA organization must demonstrate that the additional criteria provide clinical benefits that are highly likely to outweigh any clinical harms, including from delayed or decreased access to items or services;
- (B) NCDs or LCDs include flexibility that explicitly allows for coverage in circumstances beyond the specific indications that are listed in an NCD or LCD; or
- (C) There is an absence of any applicable Medicare statutes, regulations, NCDs or LCDs setting forth coverage criteria.

The following hierarchy is used to determine Medicare Advantage Medical Policy:

1. CMS Coverage Manuals or other CMS-Based Resource: Coverage provisions in interpretive manuals are instructions that are used to further define when and under what circumstances items or services may be covered (or not covered)
2. National Coverage Determinations (NCD)
3. Local Coverage Determinations (LCD)
4. Local Coverage Articles (LCA), when used in conjunction with LCD
5. EviCore's evidence-based guidelines or the appropriate alternative guideline utilized by a program/health plan in place of EviCore's guidelines.
 - Rationale supporting the adoption and use of internally developed coverage guidelines: When coverage criteria are not fully established (as defined in 42 CFR 422.101(6) (i)) in Medicare policy, an NCD, or an LCD, CMS allows a Medicare Advantage Organization (MAO) to create and use internally developed coverage criteria. See 'Coverage criteria are not fully established when (42 CFR 422.101(6) (i) above on page 3 for the full definition of 'not fully established'.
 - EviCore will exhaust all traditional Medicare policies, NCDs, and LCDs prior to using internal criteria policies. When an internally created coverage policy is utilized for a specific condition where a Medicare policy, an NCD, or LCD is not fully established, the general provisions supplemented by the internal criteria will include when the specific clinical presentation of the enrollee, for the request under review is not addressed in the Medicare coverage policy; or there are no specific coverage criteria included in a Medicare policy because the policy provides broad provisions, which may be instructive, but are not detailed enough to be used to determine the medical necessity of the request. Additionally, internal coverage policy may be used when Medicare policy includes statements within the policy that allow for coverage beyond what is written in the Medicare policy.
 - Under the guidance outlined above EviCore's evidence-based guidelines or the appropriate alternative guideline utilized by a program/health plan in place of EviCore's guidelines, EviCore may apply internally developed clinical coverage guidelines to the request under review. Each EviCore clinical coverage policy is developed following an objective, evidence-based process based on scientific

evidence, generally accepted and current standards of medical practice, and authoritative clinical practice guidelines. Each coverage policy developed and used outlines clinical benefits, addresses any clinical harm, and access to services. In addition, each member's unique clinical situation is considered in conjunction with current CMS guidelines and EviCore clinical coverage policy, as applicable.

Note: Where a Medicare Administrative Contractor (MAC) has adopted the Palmetto GBA MoIDX® Program's criteria for the LCDs governing molecular and genomic tests within their jurisdiction, EviCore's Laboratory Management program will follow the MoIDX criteria published by the MACs for those jurisdictions.

Medicare/Medicaid Dual Membership

Hierarchy and Application for dual eligible Medicare/Medicaid

Individuals enrolled in both Medicare and Medicaid are considered to be dually eligible. For individuals with both Medicare and Medicaid, the following hierarchy should be applied.

1. CMS Coverage Manuals
2. National Coverage Determinations (NCD)
3. Local Coverage Determinations (LCD)
4. Local Coverage Articles (LCA) – when used in conjunction with an LCD
5. Medicaid Coverage Policies (if Medicare/Medicaid (MMP) or Medicare/Medicaid Special Needs plans)
6. Evidence based clinical policies (EviCore) or the appropriate alternative guideline utilized by a program/health plan in place of EviCore's guidelines

Guideline Definitions

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Experimental, Investigational, or Unproven

Certain studies, treatments, procedures, or devices may be considered experimental, investigational, or unproven for **any** condition, illness, disease, injury being treated if one of the following is present:

- if there is a paucity of supporting evidence;
- if the evidence has not matured to exhibit improved health parameters;
- if clinical utility has not been demonstrated in any condition; OR
- the study, treatment, procedure, or device lacks a collective opinion of support.

Supporting evidence includes standards that are based on credible scientific evidence published in peer-reviewed medical literature (such as well conducted randomized clinical trials or cohort studies with a sample size of sufficient statistical power) generally recognized by the relevant medical community. Collective opinion of support includes physician specialty society recommendations and the views of physicians practicing in relevant clinical areas when physician specialty society recommendations are not available.

Medically Necessary

Healthcare services or supplies needed to diagnose, treat, or evaluate a condition or prevent an injury, illness, condition or disease that meets accepted standards of medicine based on evidenced-based clinical standards of care based on supporting evidence and/or collective opinion of support that is:

- Clinically appropriate in terms of type, frequency, extent, site, and duration, and considered effective for the individual's illness, injury or disease;
- Clinical utility of the technology, drug, device, treatment or procedure has been demonstrated for a diagnosis, treatment, evaluation or prevention of an illness, condition or disease based on evidence-based clinical standards of care;
- Not more costly than an alternative service or sequence of services that are at least as likely to produce equivalent therapeutic or diagnostic results as to the diagnosis or treatment of a condition;
- Not primarily for the convenience of the individual seeking medical services, health care provider, or other physicians or healthcare providers;
- Supporting evidence and/or collective opinion of support does not demonstrate that there is an alternative that is more appropriate/effective for diagnosis, treatment, or evaluation of a condition.

Supporting evidence includes standards that are based on credible scientific evidence published in peer-reviewed medical literature (such as well conducted randomized clinical trials or cohort studies with a sample size of sufficient statistical power) generally recognized by the relevant medical community. Collective opinion of support includes physician specialty society recommendations and the views of physicians practicing in relevant clinical areas when physician specialty society recommendations are not available. Determination of medical necessity is based on specific clinical guidelines.

Not Medically Necessary

Certain studies, treatments, procedures, or devices may be considered not medically necessary if there is supporting evidence but one of the following is present:

- Not clinically appropriate in terms of type, frequency, extent, site, and duration, and/or not considered effective for the individual's illness, injury or disease;
- Clinical utility of the technology, drug, device, treatment or procedure has not been demonstrated for a diagnosis, treatment, evaluation or prevention of the specific illness, condition or disease based on evidence-based clinical standards of care;
- More costly than an alternative service or sequence of services that are at least as likely to produce equivalent therapeutic or diagnostic results as to the diagnosis or treatment of a condition;
- Primarily for the convenience of the individual seeking medical services, health care provider, or other physicians or healthcare providers;
- Supporting evidence and/or collective opinion of support demonstrates that there is an alternative that is more appropriate/effective for diagnosis, treatment, or evaluation of a condition.

Supporting evidence includes standards that are based on credible scientific evidence published in peer-reviewed medical literature (such as well conducted randomized clinical trials or cohort studies with a sample size of sufficient statistical power) generally recognized by the relevant medical community. Collective opinion of support includes physician specialty society recommendations and the views of physicians practicing in relevant clinical areas when physician specialty society recommendations are not available. Determination of not medically necessary is based on specific clinical guidelines.

Special Considerations for Laboratory Testing

Laboratory-based testing is defined in terms of both the underlying technology used and, the indication for testing (i.e.: syndrome, condition, etc.). This is due to the ubiquitous use of specific technologies in laboratory medicine.

References

1. <https://www.cigna.com/health-care-providers/coverage-and-claims/policies/medical-necessity-definitions>.

2. Medically necessary - Glossary | HealthCare.gov**3. H-320.953 Definitions of "Screening" and "Medical Necessity" | AMA****4. Per Compliance Internal policy: Clinical Certification of Services – Initial UM 0045:**

- a. Experimental/Investigational: The use of a technology, drug, device, treatment, or procedure that has not been proven or recognized as having proven benefit in clinical medicine for any condition, illness, disease, or injury being treated.
- b. Medical Necessity: Refers to services or supplies for diagnosing, evaluating, treating or preventing an injury, illness, condition or disease, based on evidence-based clinical standards of care. Medically necessary services are accepted health care services and supplies provided by health care entities, appropriate to evaluation and treatment of a disease, condition, illness or injury and consistent with the applicable standard of care. Determination of medical necessity is based on specific clinical guidelines. (NCQA 2025 Standards; CMS; American College of Medical Quality)

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Guideline Development (Preface-1)

Guideline

Guideline Development (Preface-1.1)

Preface to the Imaging Guidelines

Guideline Development (Preface-1.1)

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- These evidence-based, proprietary clinical guidelines evaluate a range of advanced imaging and procedures, including NM, US, CT, MRI, PET, Radiation Oncology, Sleep Studies, Gastrointestinal Endoscopy, as well as Cardiac, musculoskeletal and Spine interventions.
- EviCore reserves the right to change and update the guidelines. The guidelines undergo a formal review annually. These clinical guidelines are based on current evidence supported by major national and international association and society guidelines and criteria, peer-reviewed literature, major treatises as well as, input from health plans, and practicing academic and community-based physicians.
- These guidelines are not intended to supersede or replace sound medical judgment, but instead, should facilitate the identification of the most appropriate imaging or other designated procedure given the individual's clinical condition. These guidelines are written to cover medical conditions as experienced by the majority of individuals. However, these guidelines may not be applicable in certain clinical circumstances, and physician judgment can override the guidelines.
- These guidelines provide evidence-based, clinical benefits with a focus on health care quality and patient safety.
- Clinical decisions, including treatment decisions, are the responsibility of the individual and his/her provider. Clinicians are expected to use independent medical judgment, which takes into account the clinical circumstances to determine individual management decisions.

Benefits, Coverage Policies, and Eligibility Issues (Preface-2)

Guideline

Benefits, Coverage Policies, and Eligibility Issues (Preface-2.1)
References (Preface-2)

Benefits, Coverage Policies, and Eligibility Issues (Preface-2.1)

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Medicare Coverage Policies

- See EviCore's Medicaid and Medicare Hierarchy and Application.

Investigational and Experimental Studies

- See EviCore's Guideline Definitions.

Clinical and Research Trials

- Similar to investigational and experimental studies, clinical trial imaging requests are reviewed to determine whether they meet Health Plan coverage and these evidence-based clinical guidelines.
- Imaging studies which are inconsistent with established clinical standards, or are requested for data collection and not used in direct clinical management are not supported.

References (Preface-2)

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1. Prospective Payment Systems - General Information. Centers for Medicare and Medicaid Services. [https://www.cms.gov/medicare/payment/prospective-payment-systems#:~:text=A%20Prospective%20Payment%20System%20\(PPS,on%20a%20predetermined%2C%20fixed%20amount](https://www.cms.gov/medicare/payment/prospective-payment-systems#:~:text=A%20Prospective%20Payment%20System%20(PPS,on%20a%20predetermined%2C%20fixed%20amount)
2. Medicares Coverage With Evidence Development: A Policy-Making Tool in Evolution. *J Oncol Pract.* 2007;3(6):296-301. doi:10.1200/jop.0763501
3. Coverage of Clinical Trials under the Patient Protection and Affordable Care Act; 42 U.S.C.A. § 300gg-8

Clinical Information (Preface-3)

Guideline

Clinical Information (Preface-3.1)

References (Preface-3)

Clinical Information (Preface-3.1)

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Clinical Documentation and Age Considerations

- These clinical guidelines use an evidence-based approach to determine the most appropriate procedure for each individual, at the most appropriate time in the diagnostic and treatment cycle. These clinical guidelines are framed by:
 - clinical presentation of the individual, rather than the studies requested
 - adequate clinical information that must be submitted to EviCore in order to establish medical necessity for advanced imaging or other designated procedures includes, but is not limited to the following:
 - Pertinent clinical evaluation since the onset or change in symptoms including a detailed history, physical examination, appropriate laboratory studies, and appropriate prior imaging studies.
 - Condition-specific guideline sections may describe additional clinical information which is required for a pertinent clinical evaluation.
 - The Spine and Musculoskeletal guidelines require x-ray studies from when the current episode of symptoms has started or changed.
 - Advanced imaging or other designated procedures should not be ordered prior to clinical evaluation of an individual by the physician treating the individual. This may include referral to a consultant specialist who will make further treatment decisions.
 - Other meaningful technological contact (telehealth visit, telephone or video call, electronic mail or messaging) since the onset or change in symptoms by an established individual can serve as a pertinent clinical evaluation.
 - Some conditions may require a face-to-face evaluation as discussed in the applicable condition-specific guideline sections.
 - A recent clinical evaluation may be unnecessary if the individual is undergoing a guideline-supported, scheduled follow-up imaging or other designated procedural evaluation. Exceptions due to routine surveillance indications are addressed in the applicable condition-specific guideline sections.
 - the evidence-based approach to determine the most appropriate procedure for each individual requires submission of medical records pertinent to the requested imaging or other designated procedures.
- Many conditions affecting the pediatric population are different diagnoses than those occurring in the adult population. For those diseases which occur in both pediatric and adult populations, minor differences may exist in management due to individual age, comorbidities, and differences in disease natural history between children and adults.

- Individuals who are 18 years old or younger should be imaged according to the Pediatric Imaging Guidelines if discussed in the condition-specific guideline sections. Any conditions not specifically discussed in the Pediatric Imaging Guidelines should be imaged according to the General Imaging Guidelines. Individuals who are >18 years old should be imaged according to the General Imaging Guidelines, except where directed otherwise by a specific guideline section.

General Imaging Information

- “Standard” or “conventional” imaging is most often performed in the initial and subsequent evaluations of malignancy. Standard or conventional imaging includes plain film, CT, MRI, or US.
 - Often, further advanced imaging is needed when initial imaging, such as ultrasound, CT, or MRI does not answer the clinical question. Uncertain, indeterminate, inconclusive, or equivocal may describe these situations.
- Appropriate use of contrast is a very important component of evidence-based advanced imaging use.
 - The appropriate levels of contrast for an examination (i.e., without contrast, with contrast, without and with contrast) is determined by the evidence-based guidance reflected in the condition-specific guideline sections.
 - If, during the performance of a non-contrast imaging study, there is the unexpected need to use contrast in order to evaluate a possible abnormality, then that is appropriate.

Ultrasound

- Diagnostic ultrasound uses high-frequency sound waves to evaluate soft tissue structures and vascular structures utilizing grey scale and Doppler techniques.
- Ultrasound allows for dynamic real-time imaging at the bedside.
 - Ultrasound is limited in areas where there is dense bone or other calcification.
 - Ultrasound also has a relatively limited imaging window so may be of limited value in evaluating very large abnormalities.
 - In general, ultrasound is highly operator-dependent, and proper training and experience are required to perform consistent, high-quality evaluations.
- Indications for ultrasound may include, but are not limited to, the following:
 - Obstetric and gynecologic imaging
 - Soft tissue and visceral imaging of the chest, abdomen, pelvis, and extremities
 - Brain and spine imaging when not obscured by dense bony structures
 - Vascular imaging when not obscured by dense bony structures
 - Procedural guidance when not obscured by dense bony structures
 - Initial evaluation of ill-defined soft tissue masses or fullness and differentiating adenopathy from mass or cyst. Prior to advanced imaging, ultrasound can be